

contact G.E.H.A. at (800) 821-6136. Authorization of a fertility medication does not imply authorization of any related fertility procedures.

Assisted Reproductive Technologies (ART) (High Option)

Assisted Reproductive Technologies (ART) are considered medically necessary for the following conditions:

- A. Unexplained infertility
- B. Diminished ovarian reserve
- C. Tubal factor infertility
- D. Male factor infertility
- E. Endometriosis
- F. Ovulatory dysfunction
 - 1. When ovulation induction has not resulted in conception
 - 2. Poor response to ovulation induction
 - 3. Hyper-response to ovulation induction where there is a risk for ovarian hyperstimulation or a multiple gestation
- G. Failure to achieve conception after 3 unsuccessful attempts with IVI, IUI or ICI treatment.
- H. Iatrogenic Infertility (High Option)

When Assisted Reproductive Technologies (ART) are not covered:

- A. When using autologous oocytes in the setting of a very poor or futile prognosis; OR
- B. When there is a failure to respond to ovarian stimulation (e.g., as demonstrated by failure to achieve at least 1 follicle >12 mm in diameter); OR
- C. ART cycle does not demonstrate the attainment of at least 1 embryo suitable for transfer; OR
- D. Lack of viable spermatozoa; OR
- E. Ovarian failure where a couple is attempting conception with their own gametes; OR
- F. Recurrent aneuploidy or ≥ 5 unexplained losses; OR
- G. Greater than 2 consecutive ART cycles without adequate egg quality or production, fertilization and/or embryo quality or development

Intracytoplasmic Sperm Injection (ICSI) (High Option)

Intracytoplasmic Sperm Injection (ICSI) ICSI is considered medically necessary for the following:

- A. Male factor infertility
 - 1. "Male factor" infertility is an alteration in sperm concentration and/or motility and/or morphology in at least two sperm analyses, collected 4 weeks apart.
- B. After failed conventional insemination